

WEIGHT LOSS

Lipotropic & B12 Injection Protocol

MIC, B-complex and lipotropic blends: technique, frequency limits and honest claims

DOCUMENT WL-008	VERSION 1.0	EFFECTIVE January 2026	REVIEW CYCLE Annual
APPLIES TO Clinical staff administering injections	OWNER Medical Director	CLASSIFICATION Confidential	SUPERSEDES All prior versions

1 Purpose & Scope

This protocol covers lipotropic blends, vitamin B12 and B-complex injections offered alongside or independently of pharmacologic weight management. The clinical risk is modest; the regulatory risk sits almost entirely in how these services are described and advertised.

THE RISK HERE IS THE CLAIM

Evidence that lipotropic injections produce meaningful weight loss independent of diet is limited. Advertising them as a fat-burning or weight-loss treatment is where practices get into difficulty — with advertising regulators and, in several states, with licensing boards. Sell what you can substantiate.

2 Formulations & Indications

Component	Typical rationale	Documentation standard
Cyanocobalamin or methylcobalamin (B12)	Treatment of documented deficiency; supplementation where absorption is impaired.	Deficiency-based use should be supported by a lab result. Non-deficiency use is elective and must be described as such.
Methionine, inositol, choline (MIC)	Marketed as supporting fat metabolism.	Evidence for independent weight-loss effect is limited. Counsel and advertise accordingly.
B-complex	General supplementation.	Elective. No weight-loss claim should be attached.
L-carnitine	Marketed as supporting fat utilization.	Evidence limited. Same counseling standard.

- The formulation used, its components and concentrations, must be recorded and approved by the medical director.
- Compounded blends are subject to the sourcing and documentation requirements in WL-006.
- Any change of formulation or supplier requires re-approval, not a substitution at the shelf.

3 Screening & Administration

- ☐ Allergy screen including sensitivity to any component and to preservatives.
- ☐ Pregnancy and breastfeeding status.

- ☐ Renal or hepatic impairment, which may affect suitability.
- ☐ Current medications and supplements, to avoid duplication and interaction.
- ☐ For B12: relevant lab result where use is deficiency-based.
- ☐ Consent obtained, including an explicit statement of what the injection is and is not expected to achieve.

- 1 **Verify the product**, concentration, lot and expiry or beyond-use date against the order.
- 2 **Inspect the vial** for particulates, discoloration or damage. Discard if in doubt.
- 3 **Select the site** — deltoid, ventrogluteal or vastus lateralis — appropriate to the volume being given.
- 4 **Prepare aseptically** and administer by intramuscular injection using appropriate needle length for the patient's build.
- 5 **Rotate sites** between doses and record the site used each time.
- 6 **Observe** for the practice's standard post-injection interval; a first dose warrants a longer observation.
- 7 **Record** product, lot, dose, site, administering clinician and patient response.

FREQUENCY LIMITS

Set a maximum frequency per formulation, approved by the medical director, and apply it consistently. Weekly is common for elective supplementation. Escalating frequency because a patient wants faster results has no clinical basis and undermines the position that these are medically supervised services.

4 Counseling & Marketing Standards

What to say

- Describe the components and their generally understood role plainly.
- State clearly where evidence for a weight-loss effect is limited.
- Position these injections as adjunctive to nutrition and activity, never as a substitute for either.
- Where used for documented B12 deficiency, explain that specifically — this is the one clearly evidence-supported indication in the group.
- Set expectations before the first injection, not after a course has been purchased.

What not to say or publish

- Claims that the injection burns fat, melts fat, or causes weight loss on its own.
- Specific pounds-lost figures attributed to the injection.
- Metabolism-boosting claims presented as established fact.
- Energy-level claims presented as guaranteed outcomes.
- Before-and-after imagery attributing results to the injection where the patient was also on pharmacotherapy or a structured diet.
- Package deals framed around a promised amount of weight loss.

CROSS-ATTRIBUTION

Showing a before-and-after from a patient on semaglutide while marketing lipotropic injections attributes one treatment's result to another. It is deceptive even if both images are real and unretouched. Attribute results to the treatment that produced them.

5 Documentation

- Formulation, components and concentration, with medical director approval on file.
- Indication for this patient, and any supporting lab result.
- Consent including the expectation-setting discussion.
- Every administration: date, product, lot, expiry, dose, site, clinician.
- Frequency tracked against the approved maximum.
- Adverse reactions and actions taken.
- Marketing materials reviewed against the claim standards above and re-reviewed annually.

6 Quick Reference

QUICK REFERENCE CARD

Lipotropic & B12 injections

1. **Verify product, lot, concentration, expiry** against the order.

2. **Allergy, pregnancy, renal and medication screen** every time.

3. **Aseptic IM technique**, rotate sites, record the site.

4. **Observe** after injection — longer for a first dose.

5. **Respect the approved frequency limit.** No escalation on request.

6. **Say what is evidence-supported.** B12 deficiency is; fat-burning is not.

7. **Never attribute another treatment's results** to these injections.

Print, laminate and post at the point of care

7 Medical Director Authorization

This protocol is adopted as a standing order for the practice named below. By signing, the medical director confirms the protocol has been reviewed against current clinical guidance, applicable state scope-of-practice rules, and the training level of the staff authorized to perform it.

PRACTICE NAME	LOCATION / SITE
MEDICAL DIRECTOR — PRINTED NAME & CREDENTIALS	LICENSE NUMBER & STATE
SIGNATURE	DATE ADOPTED / NEXT REVIEW DATE

Template notice. This document is an editable template provided for educational and operational guidance. It is not legal advice, does not constitute a physician–patient relationship, and does not guarantee regulatory compliance or the outcome of any inspection. Drug doses, concentrations and device settings must be verified against current manufacturer labeling and prevailing clinical guidance before use. A licensed medical director must review, customize and sign this protocol before it is placed into service, and must confirm that every task assigned within it falls inside the scope of practice of the personnel performing it in your state.

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